

SENATE BILL 2149

By Walley

AN ACT to amend Tennessee Code Annotated, Title 9;  
Title 33; Title 53; Title 63 and Title 68, relative to  
clinical trials.

WHEREAS, ibogaine is a naturally occurring psychoactive compound that has demonstrated promise in treating a wide range of mental health and neurological conditions, including opioid use disorder, post-traumatic stress disorder (PTSD), traumatic brain injury, depression, anxiety, multiple sclerosis, and more; and

WHEREAS, ibogaine is classified as a Schedule I drug by the federal government, making research and access difficult, and forcing patients to seek unregulated treatment abroad; and

WHEREAS, multiple states have authorized and funded research into ibogaine, and Texas has launched a research consortium facilitating a multistate effort to conduct clinical trials on ibogaine as a medical treatment; and

WHEREAS, recent studies demonstrate ibogaine's ability to significantly reduce symptoms of substance use addiction, PTSD, depression, and anxiety after one treatment; and

WHEREAS, ibogaine treatment has been associated with marked improvements in patients with traumatic brain injury, with results that include significant improvements in concentration, information processing, memory, and impulse control; and

WHEREAS, Stanford University has studied the impact of treatment on veterans who have been supported by nonprofit organizations that facilitate veterans traveling to ibogaine retreats abroad with reported substantial improvements in depression scores, PTSD symptoms, anxiety, sleep, quality of life, substance abuse, and emotional well-being; and

WHEREAS, trusted institutions, including the federal Food and Drug Administration and Department of Veterans Affairs, are actively supporting and investing in research on psychedelic-assisted therapies for PTSD and related mental health conditions; and

WHEREAS, expediting research into the medical potential of ibogaine is an effective and safe way to ensure patients have access to a potentially groundbreaking treatment; now, therefore,

BE IT ENACTED BY THE GENERAL ASSEMBLY OF THE STATE OF TENNESSEE:

SECTION 1. Tennessee Code Annotated, Title 33, is amended by adding the following as a new chapter:

**33-12-101. Short title.**

This chapter is known and may be cited as the "Helping Open Pathways to Effective (HOPE) Treatment Act."

**33-12-102. Chapter definitions.**

As used in this chapter:

- (1) "Commissioner" means the commissioner of mental health and substance abuse services;
- (2) "Department" means the department of mental health and substance abuse services;
- (3) "Drug developer" means a pharmaceutical company, biotechnology company, or contract development and manufacturing organization engaged in drug development and manufacturing;
- (4) "Hospital" means any institution, place, building, or agency represented and held out to the general public as ready, willing, and able to furnish care, accommodations, facilities, and equipment for the use, in connection with the services of a physician or dentist, of one (1) or more

nonrelated persons who may be suffering from deformity, injury, or disease or from any other condition for which nursing, medical, or surgical services would be appropriate for care, diagnosis, or treatment;

(5) "Ibogaine" means ibogaine and ibogaine-based therapeutics, including ibogaine analogs; and

(6) "Institution of higher education" means any postsecondary institution operated by the board of trustees for the University of Tennessee system, the board of regents, or a state university board that offers courses of instruction leading to a certificate or degree.

**33-12-103. Establishment of cohort.**

(a) A cohort may be established under this section and apply for department selection under this chapter to conduct drug development clinical trials with ibogaine and secure the United States food and drug administration's approval of ibogaine as a medication for the treatment of:

- (1) Opioid use disorder;
- (2) Co-occurring substance use disorder; and
- (3) Any other neurological or mental health condition for which ibogaine demonstrates efficacy.

(b) A cohort established under this section must include one (1) or more of each of the following entities:

- (1) A drug developer;
- (2) A research institution, which may be an institution of higher education; and
- (3) A hospital.

**33-12-104. Lead institution - Administration - Personnel.**

(a) A cohort established under this chapter shall select a project lead from among the cohort's members to represent the cohort and perform administrative functions under this chapter, including contracting with and reporting to the department as required by this chapter. It is the intent of the general assembly that the research institution be selected as project lead.

(b) A project lead selected by the department under this chapter may employ personnel, including clinical, administrative, and data management personnel, necessary to support any cohort member's activities related to drug development clinical trials conducted under this chapter. Such activities may include participation in the Texas consortium for the purpose of clinical trial collaboration.

**33-12-105. Cohort proposal.**

(a) The cohort project lead shall submit to the department a proposal and request for funding on behalf of the cohort for purposes of conducting ibogaine drug development clinical trials in accordance with this chapter.

(b) A proposal submitted under subsection (a) must provide:

(1) The identity of all cohort members;

(2) A detailed description of the planned strategy for obtaining approval for the drug development clinical trials from the United States food and drug administration;

(3) A detailed drug development clinical trial design that includes:

(A) A description of the composition of the cohort's drug development clinical trial team and the expertise of the team members;

(B) A drug development clinical trial participant recruitment plan;

(C) Patient screening criteria and cardiac safety protocols;

(D) Administration protocols;

(E) An aftercare and post-acute treatment support plan; and

(F) A data integrity plan;

(4) A detailed plan to seek a breakthrough therapy designation for ibogaine from the United States food and drug administration under 21 U.S.C. § 356;

(5) A proposal to recognize this state's commercial interest in all intellectual property that may be generated over the course of the drug development clinical trials, including:

(A) The treatment that is the subject of the trials;

(B) Administration protocols;

(C) Treatment models or techniques; and

(D) Technology used in the trials;

(6) A plan to establish a corporate presence in this state and to promote and maintain ibogaine-related biomedical research, development, treatment, manufacturing, and distribution in this state;

(7) A plan to secure third-party payor approval for ibogaine treatment following approval by the United States food and drug administration through:

(A) Private insurers;

(B) Medicare;

(C) Medicaid; and

(D) The TRICARE program of the United States department of defense;

(8) A plan to ensure ibogaine treatment access to uninsured individuals following approval by the United States food and drug administration;

(9) A plan to train and credential medical providers to administer ibogaine treatment according to developed clinical standards; and

(10) Financial disclosures that verify the cohort's capacity to fully match state funding with funds received from non-state sources.

**33-12-106. Department selection.**

The department, in the department's sole discretion, shall select a cohort established in accordance with § 33-12-103 for the purpose of conducting ibogaine drug development clinical trials under this chapter.

**33-12-107. Contract with lead institution.**

(a) As soon as practicable after selecting a cohort to conduct ibogaine drug development clinical trials under § 33-12-106, the department shall enter into an interagency agreement with the project lead of the selected cohort to provide funding to implement the cohort's proposed ibogaine drug development clinical trials.

(b) The interagency contract described by subsection (a) must specify:

(1) The goals and objectives of the proposed ibogaine drug development clinical trials;

(2) The proposed budget;

(3) The timeline for completing the proposed objectives;

(4) The for-profit, nonprofit, or public benefit corporate entities collaborating with the cohort in the drug development clinical trials under this chapter or a clinical trial initiated in another state, including the clinical trial conducted by the Texas consortium;

(5) The percentage of the revenue arising from the drug development clinical trials to be paid to the state; and

(6) Any other information required by the department.

(c) As soon as practicable after entering into an interagency contract under subsection (a), the department shall report the existence of the contract to the general assembly by sending notice to the chief clerks of the senate and house of representatives.

(d) The department shall not disburse funds to or for a selected cohort under the interagency contract described by subsection (a) until the cohort receives and the department verifies the receipt of matching funds from sources other than the state.

**33-12-108. Investigational new drug application.**

On the department's notification that a cohort is selected to conduct the drug development clinical trials under this chapter, a drug developer or hospital member of the selected cohort or the project lead of the cohort, as specified by written agreement of the cohort members, shall, as soon as practicable:

(1) Submit an investigational new drug application to the United States food and drug administration in accordance with 21 CFR Part 312; and

(2) Seek a breakthrough therapy designation for ibogaine from the United States food and drug administration under 21 U.S.C. § 356.

**33-12-109. Drug development clinical trial sites.**

For purposes of conducting a drug development clinical trial under this chapter, only a research institution or a hospital shall serve as a trial site.

**33-12-110. Funding - Disbursement by department.**

(a) The department and cohort members may solicit and accept gifts, grants, and donations of any kind received from sources other than the state for purposes of funding drug development clinical trials under this chapter.

(b) Disbursements of funds by the department may be made incrementally based on the completion of clearly defined objectives as negotiated in the contract

described by § 33-12-107, including verifiable documentation demonstrating the efficient expenditure of both state and matching funds.

**33-12-111. Reporting requirements.**

(a) A cohort selected to conduct ibogaine drug development clinical trials shall prepare and submit to the department quarterly:

(1) A report on the progress of the drug development clinical trials conducted under this chapter; and

(2) A financial status report, including information to verify expenditures of state funds and required matching funds.

(b) The department shall submit a report to the general assembly on the progress of the drug development clinical trials conducted under this chapter no later than December 1 of each year. The report must be submitted to the chief clerk of the senate, the chief clerk of the house of representatives, and the legislative librarian, and may be submitted by electronic means.

**33-12-112. Allocation of revenue attributable to intellectual property and other rights.**

(a) The revenue attributable to all intellectual property rights and other commercial rights arising from drug development clinical trials conducted by a cohort under this chapter during the period for which the trials are funded and any following period of commercialization shall be allocated as follows:

(1) No less than five percent (5%) to the state as specified in the contract under § 33-12-107; and

(2) The remainder to the members of the cohort in the amounts specified by written agreement of the members.



(b) For purposes of this section, intellectual property rights and other commercial rights arising from the drug development clinical trials conducted under this chapter include any of the following as related to the trials:

- (1) Intellectual property, technology, and inventions;
- (2) Patents, trademarks, and licenses;
- (3) Proprietary and confidential information;
- (4) Trade secrets, data, and databases;
- (5) Tools, methods, and processes;
- (6) Treatment models or techniques;
- (7) Administration protocols; and
- (8) Works of authorship.

**33-12-113. Use of state revenue.**

(a) The state treasurer shall deposit the revenue received under § 33-12-112(a)(1) as follows:

- (1) No less than fifty percent (50%) into the Tennessee mental health innovation fund, created by § 33-12-114; and
- (2) The remainder into the general fund.

(b) The state treasurer shall develop accounting procedures for the purpose of implementing this section.

**33-12-114. Creation of innovation fund - Purpose.**

(a) There is created within the state general fund a special account to be known as the Tennessee mental health innovation fund.

(b) Unless otherwise specified in this part, moneys realized from intellectual property and other commercial rights pursuant to § 33-12-112(a)(1) and allocated pursuant to § 33-12-113(a)(1) must be deposited in the fund and used only to implement

and administer the purposes set forth in subsection (f). In addition to appropriations made to the fund, the commissioner may accept other funds, public or private, by way of gift or grant to the fund. Any such gift or grant must be deposited into the fund to be expended in accordance with this part.

(c) The state treasurer shall invest moneys in the fund for the benefit of the fund in accordance with § 9-4-603. Interest accruing on investments and deposits of the fund must be credited to and remain part of the fund.

(d) Any unencumbered moneys and any unexpended balance of the fund remaining at the end of a fiscal year do not revert to the general fund, but must be carried forward until expended in accordance with this part. No part of the fund shall be diverted to the general fund or any other public fund.

(e) The commissioner shall administer the fund, and moneys in the fund must be expended and obligated only in accordance with this part and in accordance with appropriations made by the general assembly. All expenditures from the fund are subject to review in the form of an annual report submitted by the commissioner to the general assembly.

(f) The purpose of the Tennessee mental health innovation fund is to fund proposals from behavioral health providers to train and support clinical and supportive care staff in best practices for supporting patients who have undergone ibogaine therapy.

**33-12-115. Applicability of chapter.**

This chapter applies only if ibogaine is approved by the United States food and drug administration to treat a medical condition.

**33-12-116. Medical supervision.**

A physician licensed under title 63, chapter 6 or 9, who has prescribed ibogaine for a patient shall supervise the administration of ibogaine at a hospital or other licensed healthcare facility to ensure the patient's safety while the patient is undergoing ibogaine treatment.

**33-12-117. Administration under federal law.**

This chapter does not preclude a physician from administering ibogaine in accordance with federal law.

**33-12-118. Waiver authority - Deadline to begin accepting proposals.**

(a) If the department, or another department of state government involved in the administration of this chapter, determines that a waiver or authorization from a federal agency is necessary to implement a provision of this chapter, then the department responsible for implementing such provision shall request the waiver or authorization and may delay implementing that provision until the waiver or authorization is granted.

(b) The department shall begin accepting proposals from cohorts under this chapter no later than September 1, 2026.

SECTION 2. The headings in this act are for reference purposes only and do not constitute a part of the law enacted by this act. However, the Tennessee Code Commission is requested to include the headings in any compilation or publication containing this act.

SECTION 3. This act takes effect July 1, 2026, the public welfare requiring it.